NCCP 00614 · VERSION 4

Your treatment, explained

Carboplatin and vinorelbine · lung cancer

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, bleeding, uncontrolled vomiting or severe constipation with tummy pain. Report new numbness or weakness before the next dose. Tell staff immediately about infusion pain or swelling.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your overall plan
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

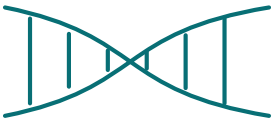
Your medicines

This chemotherapy aims to control lung cancer and symptoms when cisplatin is unsuitable. The team reviews benefit, side effects and your priorities.

VINCA ALKALOID CHEMOTHERAPY

Vinorelbine

Prescribed IV or oral route



Disrupts microtubules needed for cell division

Vinorelbine disrupts the cell structures that help divide chromosomes. This can slow cancer growth and also affect blood cells, nerves and the bowel.

Binds cell-division structures → Microtubules cannot function normally → Cell division is disrupted

What to watch for

Report fever, constipation, numbness or pain at the infusion site promptly.

Sources: nccp · smpc-vinorelbine

CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages DNA

Carboplatin forms links in DNA that hinder copying and repair. Healthy tissues can also be harmed. Its dose depends on target exposure (AUC) and measured or estimated kidney function; AUC is not a fixed number of milligrams. There are two planned doses in this regimen.

DNA → Platinum links → Copying disrupted

What to watch for

Report fever, bleeding, nausea, hearing changes or numbness. Tell the nurse immediately about possible allergy during the infusion.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your overall plan

DAYS 1 AND 8

Four 21 day cycles



Four 21 day cycles are planned, unless treatment stops or changes earlier. Day 1: carboplatin plus vinorelbine. Day 8: vinorelbine only. Your prescription specifies vinorelbine by drip OR capsules; these are alternatives, not extra doses.

NEXT PLAN AGREED

Review after treatment



Four cycles take about 12 weeks without delays. Your team agrees response scans and what happens next. Surgery or radiotherapy is not automatically included.

Four cycles may change for benefit, toxicity or patient preference.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Required safety checks are distinct from individual response imaging and follow-up. No routine tumour-marker interval is specified.

Check	Before treatment	Treatment days	Between doses	Next cycle	After treatment
Treatment	Confirm both drugs and route	Day 1both; Day 8vinorelbine	No other routine dose	Repeat up to 4 cycles	Next plan agreed
Blood tests	FBC, kidneys/liver, GFR	Pre-dose FBC; renal input	Extra tests if indicated	FBC/renal/liver each cycle	Follow abnormalities
Nerves / bowels	Baseline nerve review	Review symptoms	Report constipation/ numbness	Neuropathy each cycle	Review persistent effects

Check	Before treatment	Treatment days	Between doses	Next cycle	After treatment
Scans / other care	Disease stage and goal	No fixed scan day	As team directs	Response review	Individual next plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00614 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Carboplatin and vinorelbine · lung cancer: treatment and checks

Advanced/recurrent NSCLC when cisplatin is unsuitable

Four 21 day cycles are planned, unless treatment stops or changes earlier. Day 1: carboplatin plus vinorelbine. Day 8: vinorelbine only. Your prescription specifies vinorelbine by drip OR capsules; these are alternatives, not extra doses.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between doses	Next cycle	After treatment
Treatment NCCP 00614	Confirm both drugs and route	Day 1both; Day 8vinorelbine	No other routine dose	Repeat up to 4 cycles	Next plan agreed
Blood tests NCCP 00614	FBC, kidneys/liver, GFR	Pre-dose FBC; renal input	Extra tests if indicated	FBC/renal/liver each cycle	Follow abnormalities
Nerves / bowels NCCP 00614	Baseline nerve review	Review symptoms	Report constipation/ numbness	Neuropathy each cycle	Review persistent effects
Scans / other care NCCP 00614	Disease stage and goal	No fixed scan day	As team directs	Response review	Individual next plan

What each check is for

1

Prepare

Before first dose

This chemotherapy aims to control lung cancer and symptoms when cisplatin is unsuitable. The team reviews benefit, side effects and your priorities.

Ask / act: Confirm aim, prescription, tests and contacts.

Sources: NCCP 00614

2

Review

Before each dose

Check treatment remains appropriate

Ask / act: Report symptoms and confirm blood results before dosing.

Sources: NCCP 00614

3

Next plan

At agreed review points

Assess benefit and recovery

Ask / act: Confirm scan and follow-up dates with the team.

Sources: NCCP 00614

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

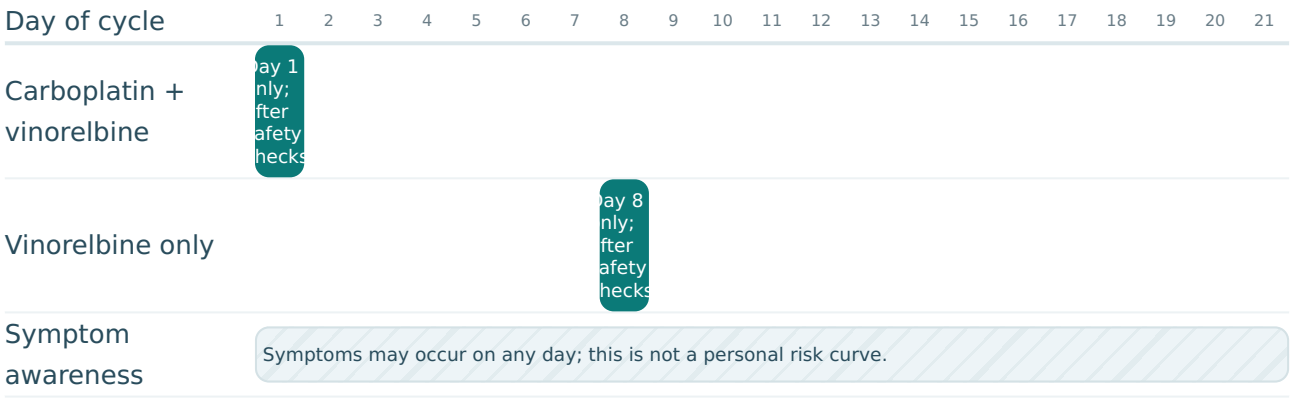
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four 21 day cycles are planned, unless treatment stops or changes earlier. Day 1: carboplatin plus vinorelbine. Day 8: vinorelbine only. Your prescription specifies vinorelbine by drip OR capsules; these are alternatives, not extra doses.



Timing varies: Tiredness, bowel changes and infection can occur between visits. Blood tests and your symptoms guide dose decisions; there is no guaranteed safe day.

Changes to the plan: The team may delay, omit or reduce doses for safety. Do not add a missed dose yourself.

Why these bands stay broad: Relative treatment days, without personal dates. Symptoms are not confined to a fixed day. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00614 v4 · official regimen ↗

There are no guaranteed “safe days”. Contact oncology immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, bleeding, uncontrolled vomiting or severe constipation with tummy pain. Report new numbness or weakness before the next dose. Tell staff immediately about infusion pain or swelling.

Read the whole cycle as text

Day 1: Carboplatin + vinorelbine

Both medicines after safety checks; confirm prescribed vinorelbine route. Your team confirms whether this dose can proceed.

Day 2–7: At home

No planned chemotherapy on these days. Use only the support medicines prescribed for today. Call using your contact plan if unwell; do not wait for a treatment day.

Day 8: Vinorelbine only

No carboplatin today. Vinorelbine by the prescribed route after safety checks. Your team confirms whether this dose can proceed.

Day 9–21: At home

No planned chemotherapy on these days. Use only the support medicines prescribed for today. Call using your contact plan if unwell; do not wait for a treatment day.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Confirm the prescribed route

Vinorelbine by drip and vinorelbine capsules are alternatives. Ask the team to mark your route.



STEP 2

Day 1 and Day 8

Day 1 includes carboplatin; Day 8 contains vinorelbine only. Arrange blood tests and clearance for each dose.



STEP 3

Capsules only when prescribed

Keep in the original packaging and follow pharmacy storage/handling instructions. Do not repeat a vomited dose or make up a missed dose.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00614 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Different support on Day 1 and Day 8

Day 1 carboplatin needs a stronger anti-sickness plan. Day 8 depends on whether vinorelbine is by drip or capsules. Use the written prescription for that day and route; do not repeat the entire Day 1course automatically.

2 Bowel plan

Vinorelbine and some anti-sickness medicines can cause constipation. Follow the prescribed bowel plan and call for severe tummy pain, swelling or vomiting.

3 If capsules are prescribed

Swallow whole with water and some food. Do not chew or open capsules. Do not take an extra dose after vomiting; contact the team for missed-dose advice. Return damaged capsules to pharmacy.

4 Check medicine interactions

The team should check all medicines, including anti-sickness treatment. Do not start new tablets, supplements or vaccines without advice.

Sources: nccp · ncis

Understanding the evidence

Carboplatin with vinorelbine has been studied in advanced lung cancer.

Historical phase II evidence used alternating IV/oral vinorelbine, not every route variant, and cannot quantify comparative benefit.

Sources: obrien-2004

PREPARE FOR THE CONVERSATION

Questions worth asking

Am I having both oral and IV vinorelbine?

No. Your written plan must identify the prescribed route for each dose. Do not treat capsule instructions as additional treatment.

Why is Day 8different?

Only vinorelbine is planned. Blood results, symptoms and the route determine whether dosing and the support plan are appropriate.

What if treatment changes?

Ask your team to update the cycle record, prescription and contact plan. A stamp is not permission for the next dose.

> **Try explaining the plan back:** Explain back your treatment days, checks and whom to contact when unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00614 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00614 v4 · official regimen

- Navelbine IV product information
- NCCP antiemetic guidance v6
- O'Brien et al.2004 carboplatin with alternating IV/oral vinorelbine
- NCCP Antiemetic Medicines for inclusion in NCIS (Medical Oncology) · v8, 10 February 2025

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6884/614_V4_Carboplatin_Vinorelbine.pdf

https://www.medicines.org.uk/emc/product/3912/smpc

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://pubmed.ncbi.nlm.nih.gov/15151949/

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf